

COMPLICAÇÕES PÓS-OPERATÓIRAS EM CIRURGIAS ABDOMINAIS



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POR ONDE COMEÇAR AO RECEBER O CASO?

Info do paciente

IOP

Instabilidade

Infusões

Perdas

Intercorrências

Sangramento e transfusão

Medicações para transporte



Sedação

Immediate interruption of sedation compared with usual sedation care in critically ill postoperative patients (SOS-Ventilation): a randomised, parallel-group clinical trial

Gerald Chanques, Matthieu Conseil, Claire Roger, Jean-Michel Constantin, Albert Prades, Julie Carr, Laurent Muller, Boris Jung, Fouad Belaïq, Moussa Cissé, Jean-Marc Delay, Audrey de Jong, Jean-Yves Lefrant, Emmanuel Futier, Grégoire Mercier, Nicolas Molinari, Samir Jaber, on behalf of the SOS-Ventilation study investigators*

Lancet Respir Med 2017

	Control group (n=68)	Intervention group (n=69)	Group difference* (95% CI)	p value
Primary outcome				
Time between randomisation and successful extubation, h	50 (29-93)	8 (4-36)	-33.6 (-44.9 to -22.4)	<0.0001
Adjusted hazard ratio (95% CI)	-	-	5.2 (3.1-8.8)	<0.0001



BNM PARCIAL

- Transporte x Tempo de ação
- - Diafragma x Periféricos
- -TOF ratio $> 0,7$ (0.9?)
- - Elevação Cabeça 5 s $\gg$ TOF 0,5-0,7

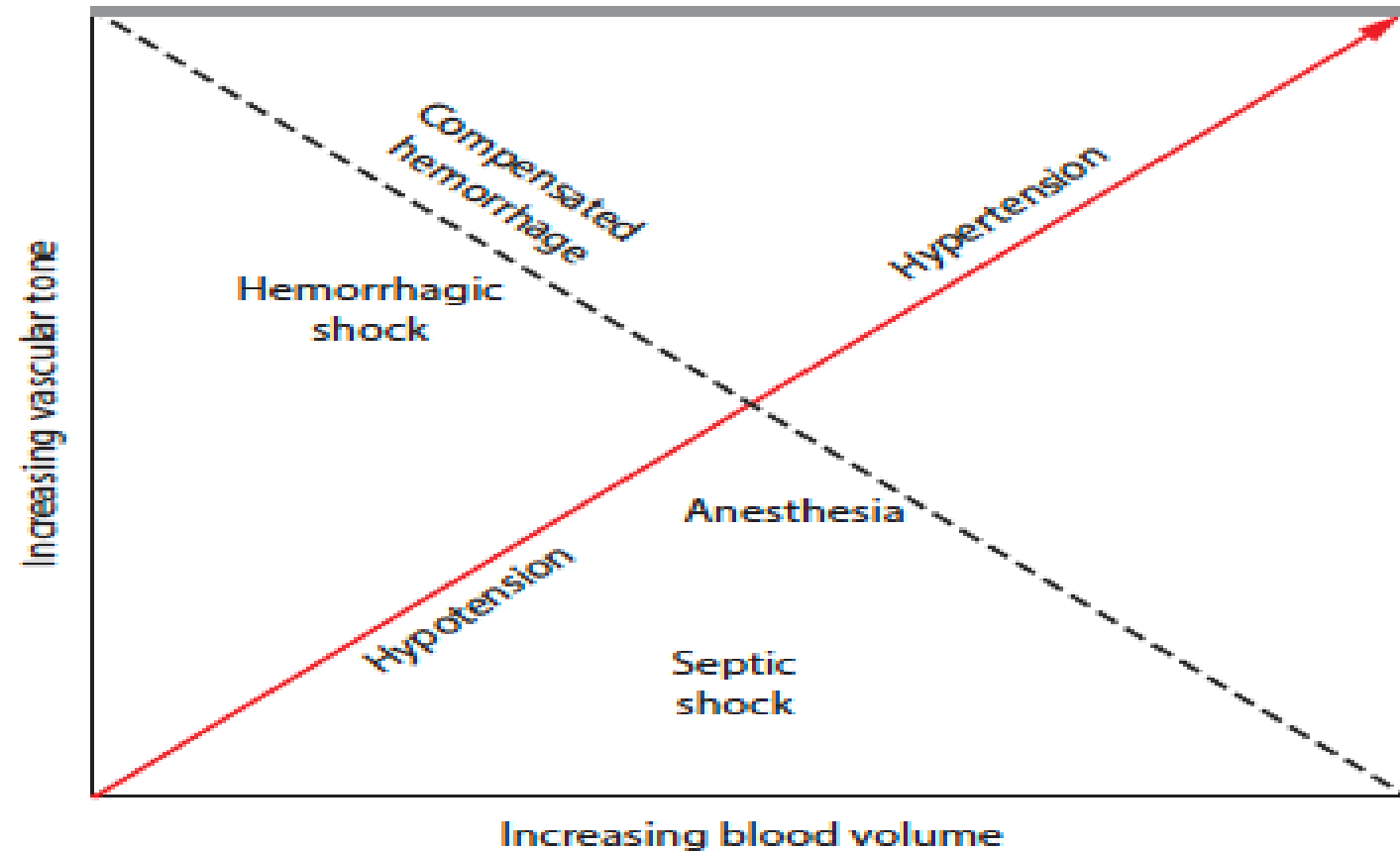


FIGURE 1. TOF scan.

two parameters of pulmonary function, that is tidal volume and vital capacity, start to recover at a TOF ratio of 0.7 [1]. Although higher degrees of neuro-



CHOQUE PÓS OPERATÓRIO



Preoperative Risk and the Association between Hypotension and Postoperative Acute Kidney Injury

Michael R. Mathis, M.D., Bhiken I. Naik, M.B.B.Ch., Robert E. Freundlich, M.D., M.S., M.S.C.I., Amy M. Shanks, Ph.D., Michael Heung, M.D., Minjae Kim, M.D., Michael L. Burns, M.D., Ph.D., Douglas A. Colquhoun, M.B. Ch.B., M.Sc., M.P.H., Govind Rangrass, M.D., Allison Janda, M.D., Milo C. Engoren, M.D., Leif Saager, M.D., M.M.M., Kevin K. Tremper, M.D., Ph.D., Sachin Kheterpal, M.D., M.B.A.* on behalf of Multicenter Perioperative Outcomes Group Investigators

Preoperative Risk

Validation Cohort				
Intraoperative Hypotension - Absolute MAP Values	Quartile 1* Low Preoperative Risk (N = 8,829)	Quartile 2* Medium Preoperative Risk (N = 9,580)	Quartile 3* High Preoperative Risk (N = 9,225)	Quartile 4* Highest Preoperative Risk (N = 8,863)
No Relative Hypotension	(reference) n = 5,041	(reference) n = 5,647	(reference) n = 5,205	(reference) n = 4,486
MAP 60-64 mmHg	0.93 (0.64-1.65) n = 1,963	1.17 (0.91-1.51) n = 2,015	1.10 (0.91-1.33) n = 2,025	1.19 (1.04-1.36) Increased AKI n = 1,981
MAP 55-59 mmHg	0.96 (0.61-1.51) n = 1,217	1.05 (0.76-1.44) n = 1,231	1.05 (0.84-1.33) n = 1,247	1.34 (1.16-1.56) Increased AKI n = 1,473
MAP 50-54 mmHg	0.72 (0.31-1.66) n = 406	1.19 (0.75-1.89) n = 456	1.46 (1.06-2.02) Increased AKI n = 441	1.69 (1.37-2.08) Increased AKI n = 538
MAP <50 mmHg	0.95 (0.35-2.60) n = 202	2.62 (1.65-4.16) Increased AKI n = 231	1.54 (1.06-2.23) Increased AKI n = 307	1.54 (1.21-1.97) Increased AKI n = 385

Hypotension
Severity

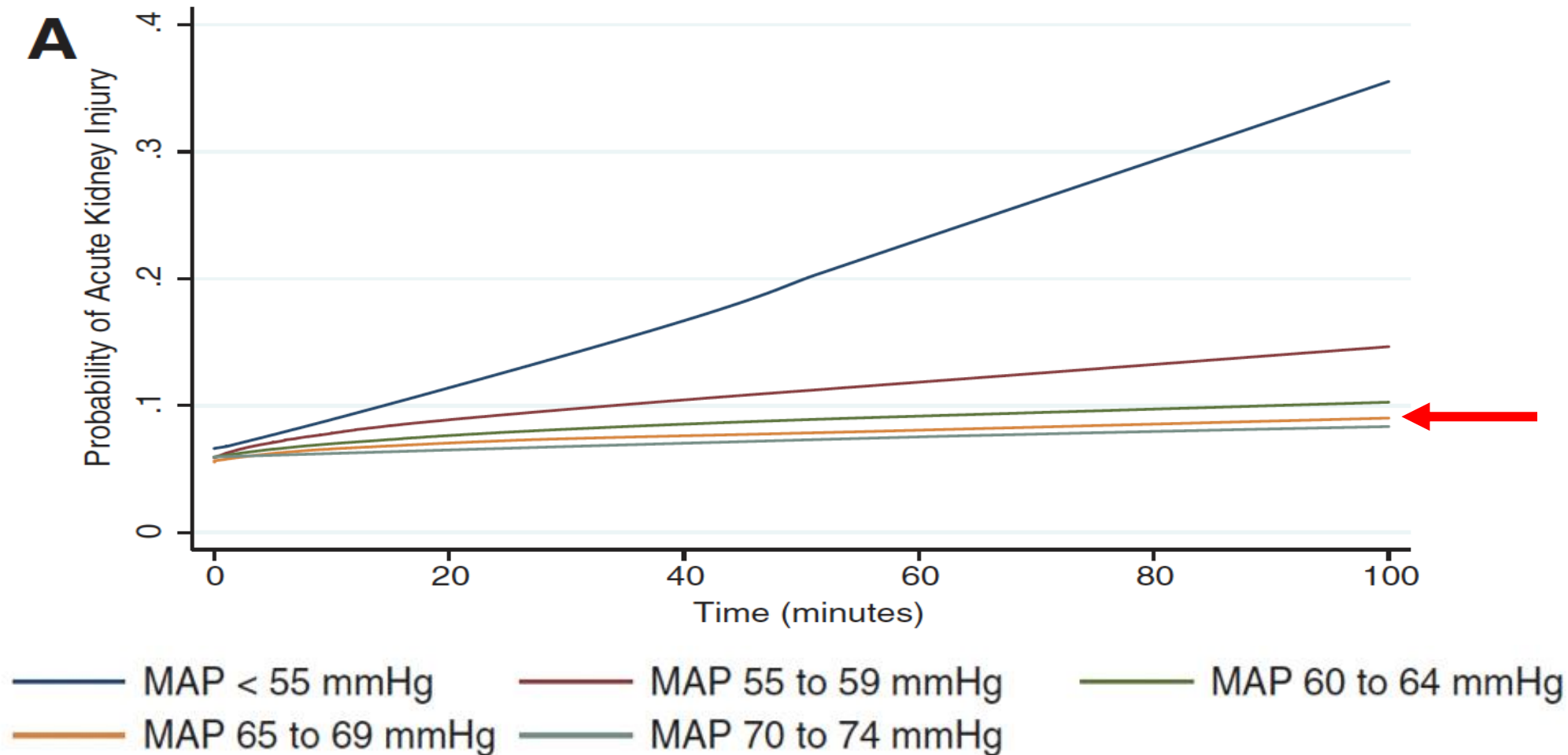


PAM 65?

Relationship between Intraoperative Mean Arterial Pressure and Clinical Outcomes after Noncardiac Surgery

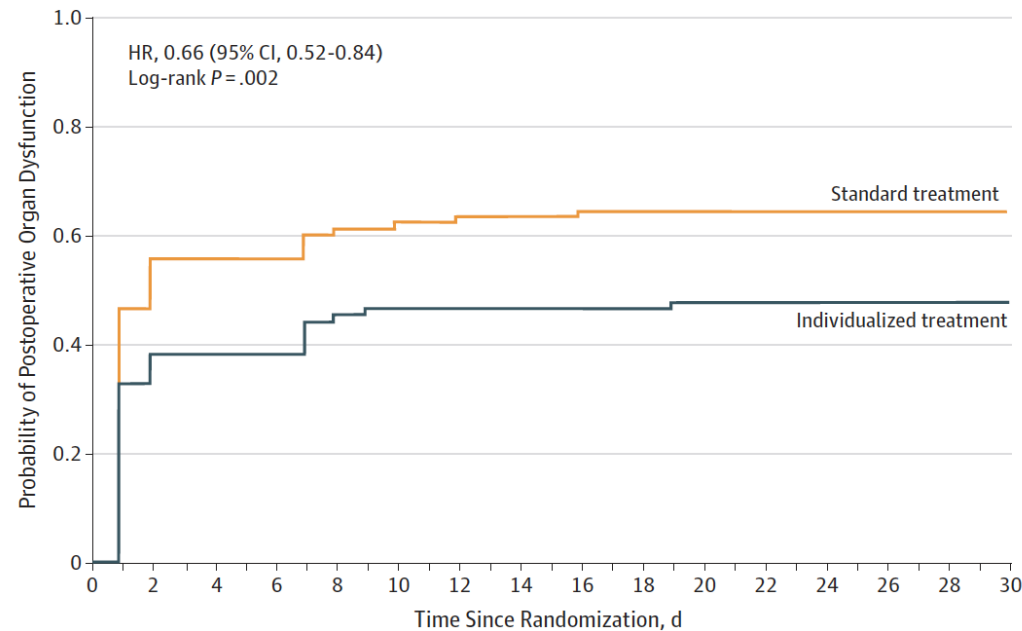
Toward an Empirical Definition of Hypotension

Michael Walsh, M.D.,* Philip J. Devereaux, M.D., Ph.D.,† Amit X. Garg, M.D., Ph.D.,‡
Andrea Kurz, M.D.,§ Alparslan Turan, M.D.,|| Reitze N. Rodseth, M.D.,# Jacek Cywinski, M.D.,**
Lehana Thabane, Ph.D.,†† Daniel I. Sessler, M.D.‡‡



PAM MAIS ELEVADA?

Figure 3. Kaplan-Meier Estimates of the Probability of Postoperative Organ Dysfunction by Day 30 After Surgery



No. at risk					
Standard treatment	145	78	65	58	54
Individualized treatment	147	99	91	82	80

JAMA | Original Investigation | CARING FOR THE CRITICALLY ILL PATIENT

Effect of Individualized vs Standard Blood Pressure Management Strategies on Postoperative Organ Dysfunction Among High-Risk Patients Undergoing Major Surgery A Randomized Clinical Trial

Emmanuel Futier, MD, PhD; Jean-Yves Lefrant, MD, PhD; Pierre-Gregoire Guinot, MD, PhD; Thomas Godet, MD, PhD; Emmanuel Lorne, MD; Philippe Cuvillon, MD, PhD; Sebastien Bertran, MD; Marc Leone, MD, PhD; Bruno Pastene, MD; Vincent Piriou, MD, PhD; Serge Molliex, MD, PhD; Jacques Albanese, MD, PhD; Jean-Michel Julia, MD; Benoit Tavernier, MD, PhD; Etienne Imhoff, MD; Jean-Etienne Bazin, MD, PhD; Jean-Michel Constantin, MD, PhD; Bruno Pereira, PhD; Samir Jaber, MD, PhD; for the INPRESS Study Group



PAM MAIS ELEVADA?

Original Contribution

Optimal blood pressure decreases acute kidney injury after gastrointestinal surgery in elderly hypertensive patients: A randomized study☆☆☆☆☆☆☆☆

Optimal blood pressure reduces acute kidney injury

Xiujuan Wu^{a,1}, Zongming Jiang^{b,1}, Jing Ying^c, Yangyang Han^d, Zhonghua Chen^{b,*}

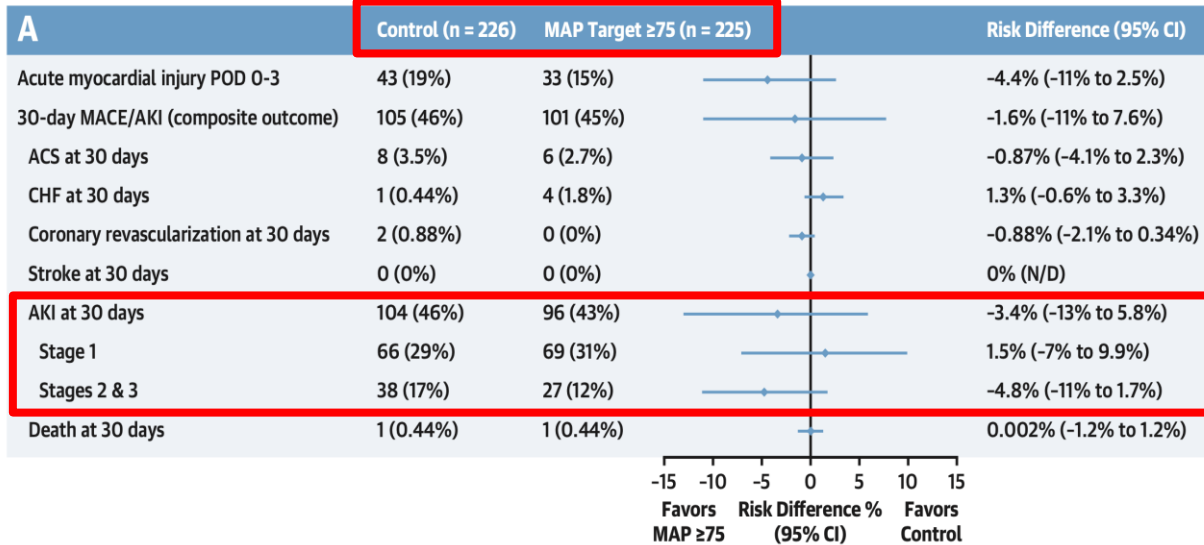
Table 3
Perioperative occurrence of AKI, and other adverse outcomes.

	I (n = 230)	II (n = 206)	III (n = 210)	P value
	65–79 mm Hg	80–95 mm Hg	and 96–110 mm Hg,	
Primary outcome				
Incidence of AKI n(%)	31 (13.5%)	13 (6.3%)	27 (12.9%)	0.033
KDIGO stage1 n(%)	20 (8.7%)	9 (4.4%)	20 (9.5%)	
KDIGO stage2 n(%)	11 (4.5%)	4 (1.9%)	7 (3.4%)	
KDIGO stage3 n(%)	0 (—)	0 (—)	0 (—)	

Targeting Higher Intraoperative Blood Pressures Does Not Reduce Adverse Cardiovascular Events Following Noncardiac Surgery

Patrick M. Wanner, MD,^{a,*} Dirk U. Wulff, PhD,^b Mirjana Djurdjevic,^a Wolfgang Korte, MD,^c Thomas W. Schnider, MD,^a Miodrag Filipovic, MD^a

FIGURE 3 Effect of Targeting Higher Intraoperative Blood Pressures on MACE



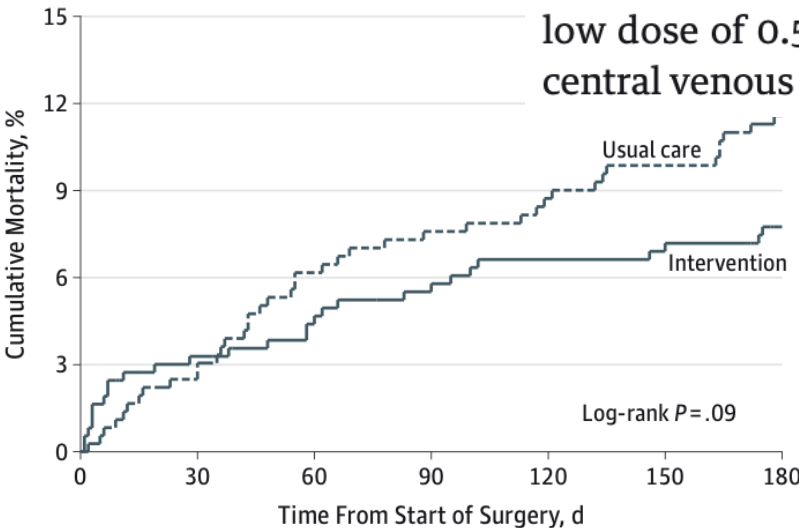
AUMENTAR DC ?

Original Investigation | CARING FOR THE CRITICALLY ILL PATIENT

Effect of a Perioperative, Cardiac Output–Guided Hemodynamic Therapy Algorithm on Outcomes Following Major Gastrointestinal Surgery A Randomized Clinical Trial and Systematic Review

Rupert M. Pearse, MD; David A. Harrison, PhD; Neil MacDonald, FRCA; Michael A. Gillies, FRCA; Mark Blunt, FRCA; Gareth Ackland, PhD; Michael P. W. Grocott, MD; Aoife Ahern, BSc; Kathryn Griggs, MSc; Rachael Scott, PhD; Charles Hinds, FRCA; Kathryn Rowan, PhD; for the OPTIMISE Study Group

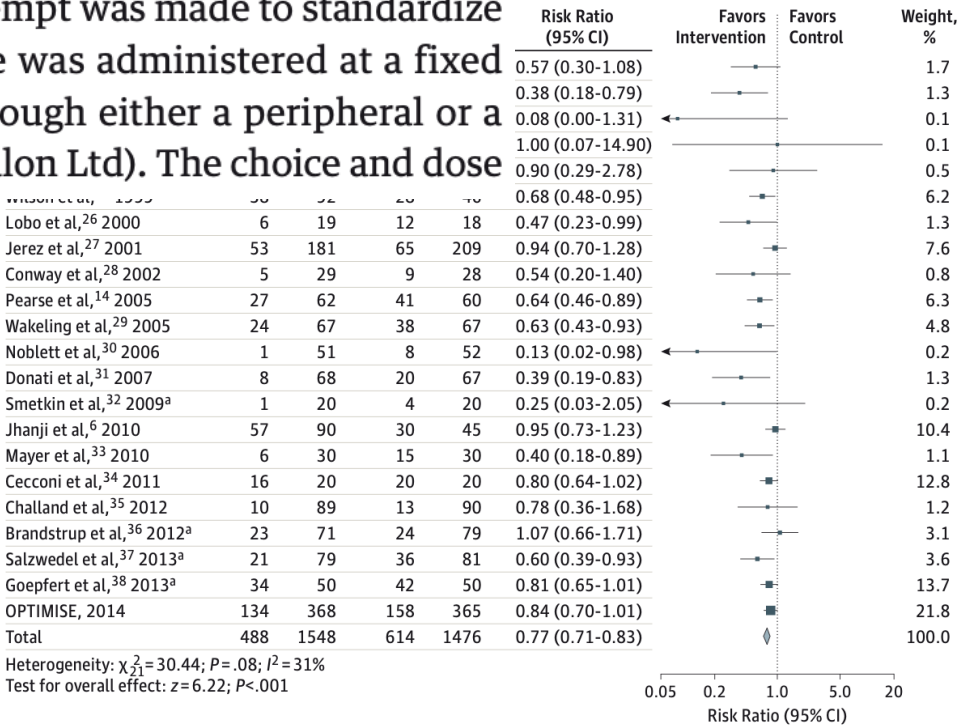
Figure 2. Cumulative Incidence of Mortality Up to 180 Days Following Major Gastrointestinal Surgery Using a Cardiac Output–Guided Hemodynamic Therapy Algorithm vs Usual Care



No. at risk							
Intervention	368	350	344	339	334	333	306
Usual care	365	348	331	325	321	317	286

safety profile.^{6,7,14-16} Intravenous colloid solution was administered in 250-mL boluses to achieve and maintain a maximal value of stroke volume; no attempt was made to standardize choice of colloid. Dopexamine was administered at a fixed low dose of 0.5 $\mu\text{g/kg/min}$ through either a peripheral or a central venous catheter (Cephalon Ltd). The choice and dose

Complications After Surgery



AUMENTAR DC ?

Cardiac output-guided haemodynamic therapy for patients undergoing major gastrointestinal surgery: OPTIMISE II randomised clinical trial

OPTIMISE II Trial Group

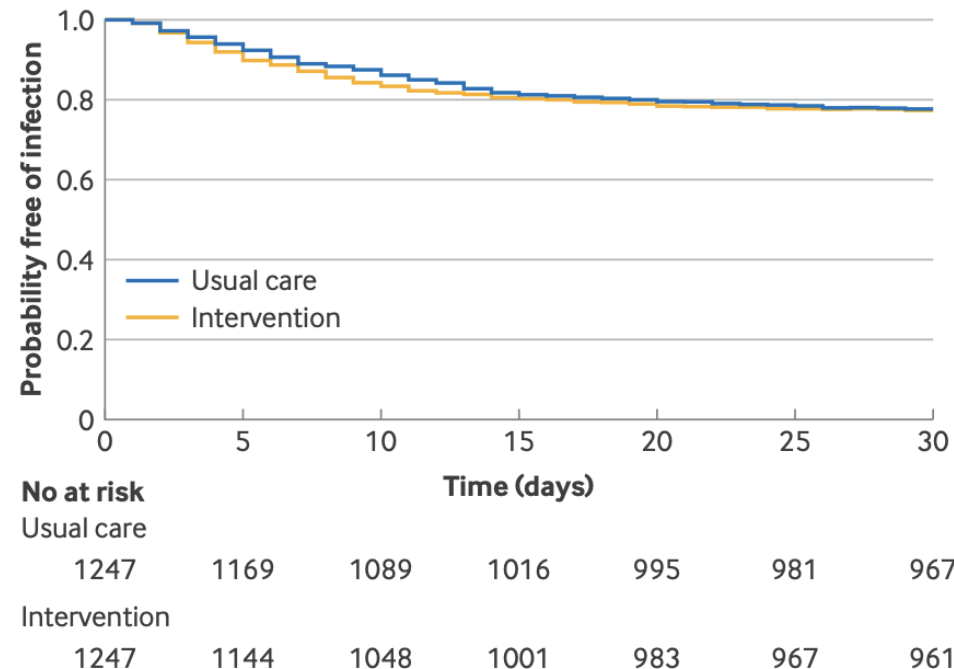
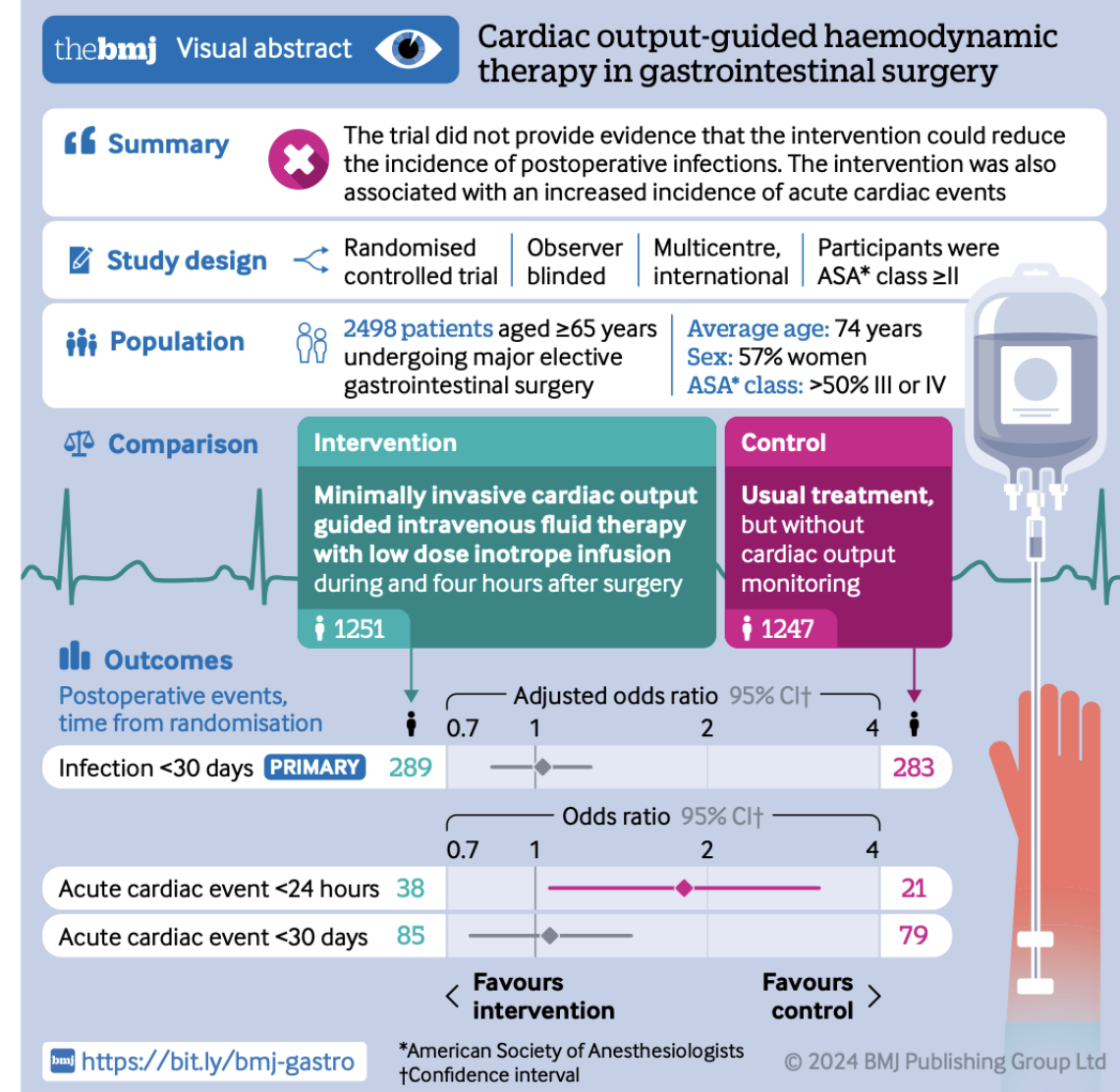


Fig 3 | Kaplan-Meier plot of time to first postoperative infection within 30 days from randomisation



VOLUME QUANTO ??

The NEW ENGLAND JOURNAL *of* MEDICINE

ESTABLISHED IN 1812

JUNE 14, 2018

VOL. 378 NO. 24

Restrictive versus Liberal Fluid Therapy for Major Abdominal Surgery

P.S. Myles, R. Bellomo, T. Corcoran, A. Forbes, P. Peyton, D. Story, C. Christophi, K. Leslie, S. McGuinness, R. Parke, J. Serpell, M.T.V. Chan, T. Painter, S. McCluskey, G. Minto, and S. Wallace, for the Australian and New Zealand College of Anaesthetists Clinical Trials Network and the Australian and New Zealand Intensive Care Society Clinical Trials Group*

Table 2. Blood Loss and Administered Intravenous-Fluid Volumes.*

Variable	Restrictive Fluid (N = 1490)	Liberal Fluid (N = 1493)	P Value
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At 24 hr after surgery

Median cumulative total for intravenous fluids (IQR) — ml	3671 (2885 to 4880)	6146 (5000 to 7410)	<0.001
Median fluid balance (IQR) — ml¶	1380 (540 to 2338)	3092 (2010 to 4241)	<0.001†
Median weight gain (IQR) — kg	0.3 (−1.0 to 1.9)	1.6 (0.0 to 3.6)	ND

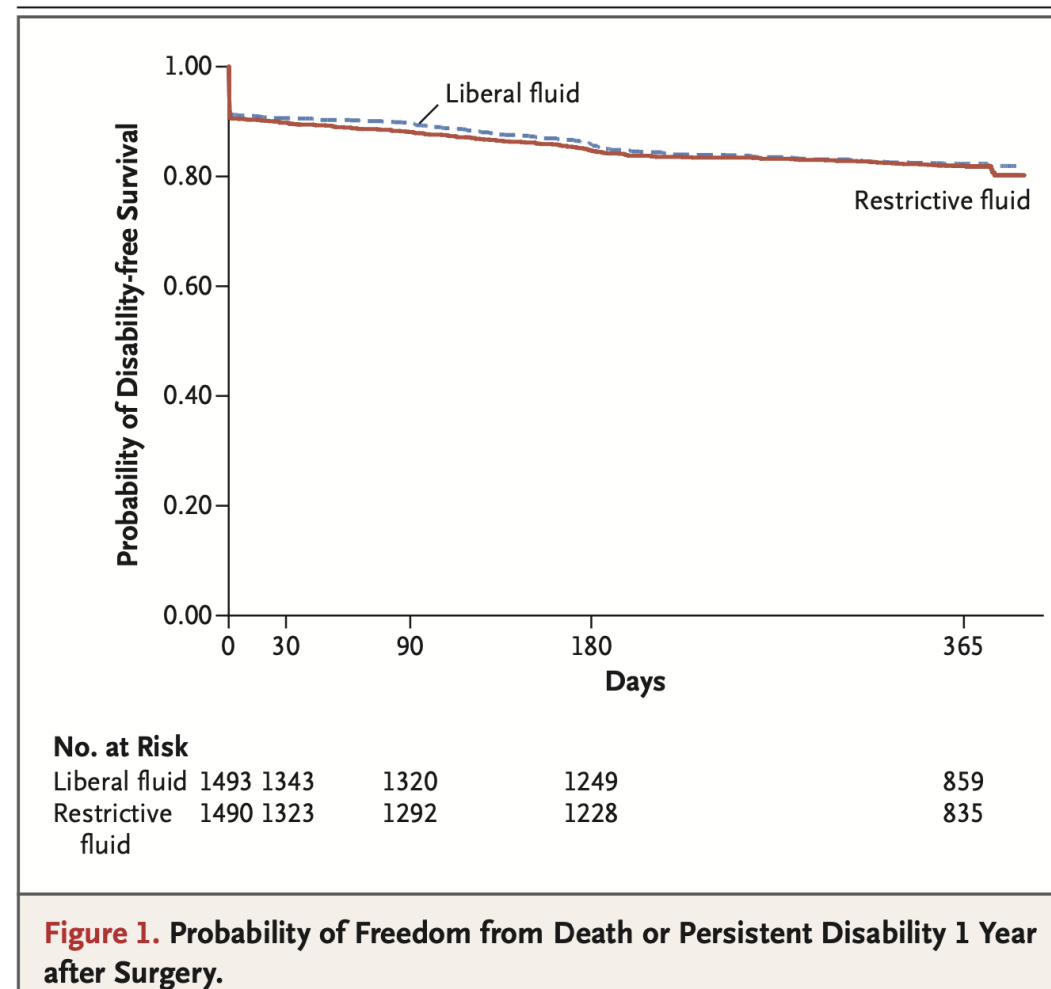
Colloid‡

500 (250 to 1000)

500 (400 to 750)

0.89

VOLUME QUANTO ??



VOLUME QUANTO ??

Table 3. Primary and Secondary Outcomes.*

Outcome	Restrictive Fluid (N=1490)	Liberal Fluid (N=1493)	Hazard or Risk Ratio (95% CI)†	P Value
Surgical-site infection — no./total no. (%)	245/1481 (16.5)	202/1487 (13.6)	1.22 (1.03–1.45)	0.02
Acute kidney injury — no./total no. (%)**	124/1443 (8.6)	72/1439 (5.0)	1.71 (1.29–2.27)	<0.001
Renal-replacement therapy — no./total no. (%)	13/1460 (0.9)	4/1462 (0.3)	3.27 (1.01–13.8)	0.048
Median peak serum lactate level (IQR) — mmol per liter††	1.6 (1.1–2.5)	1.6 (1.1–2.4)	NA	NA



GASTROPARESIA

Glicemia

Opióides

Débito de SNG

Pró-cinéticos (Antagonistas D2 x Motilina)

Distúrbios HE



<https://www.lecturio.com/magazine/stomach/>



Prokinetic therapy for feed intolerance in critical illness: One drug or two?

Nam Q. Nguyen, MBBS (Hons), FRACP; Marianne Chapman, BMBS, FANZCA, FJFICM;
Robert J. Fraser, MBBS, FRACP, PhD; Laura K. Bryant, BHSc; Carly Burgstad, BHSc (Hons);
Richard H. Holloway, MBBS, FRACP, MD

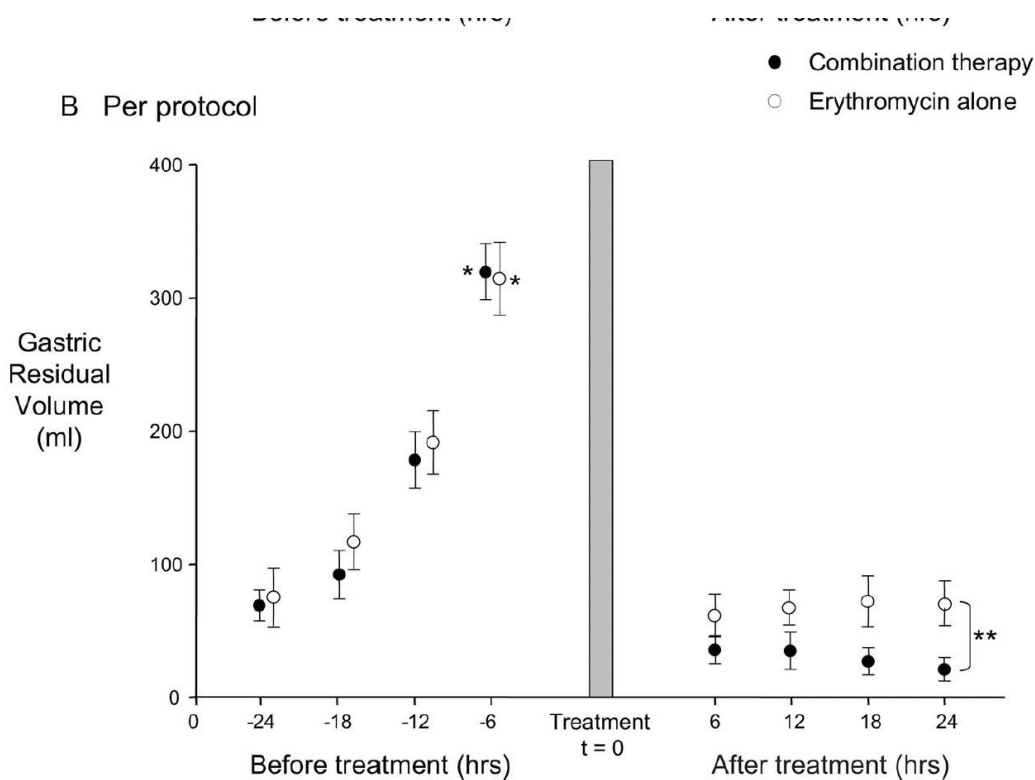


Figure 1. Six-hourly gastric residual volumes (GRVs) during the 24 hrs before (pretreatment) and after (day 1) the commencement of either combination therapy or erythromycin alone, based on intention to treat (A, including all enrolled patients, n = 75) and per-protocol (B, including only patients who completed all 7 days of the study, n = 61) analyses. * $p < .0001$, GRV 24 hrs before vs. after treatment with either combination or erythromycin-alone therapy; ** $p < .05$, erythromycin vs. combination therapy 24 hrs after treatment.

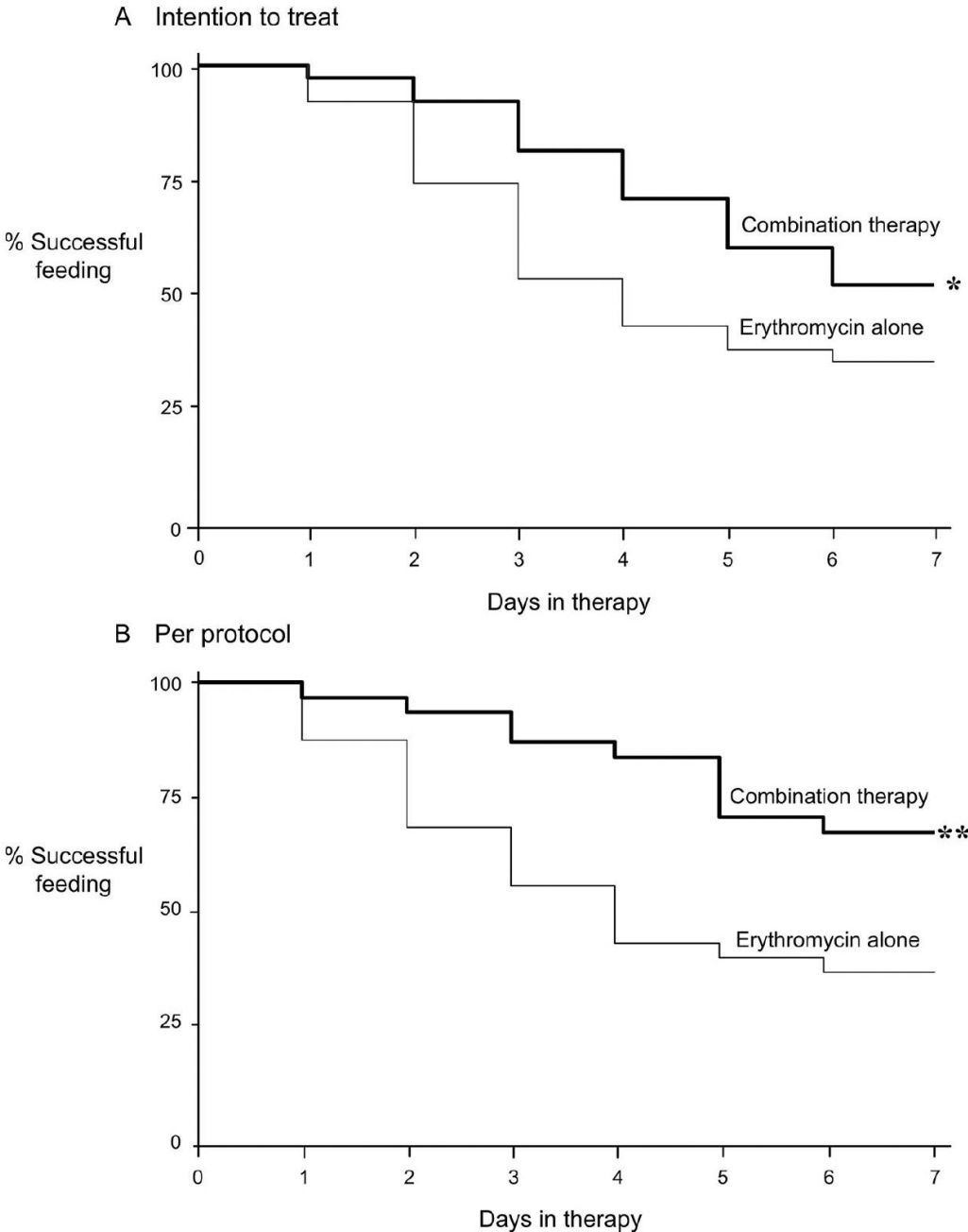


Figure 2. Kaplan-Meier plots depicting the effectiveness of combination and erythromycin-alone therapy on the success of feeding over the 7 days, based on intention to treat (A, including all enrolled patients, n = 75) and per-protocol (B, including only patients who completed all 7 days of the study, n = 61) analyses. * $p = .04$ vs. erythromycin; ** $p < .01$ vs. erythromycin.

ÍLEO PÓS OPERATÓRIO

“Fisiológico” x prolongado >> 5-6 dias ?

Eletrólitos

Opióides x Antagonistas (Alvimopan ?)

SNG – Rotina ? Eras

AINE x ERAS

Café ? Chiclete ?

Deambulação ?

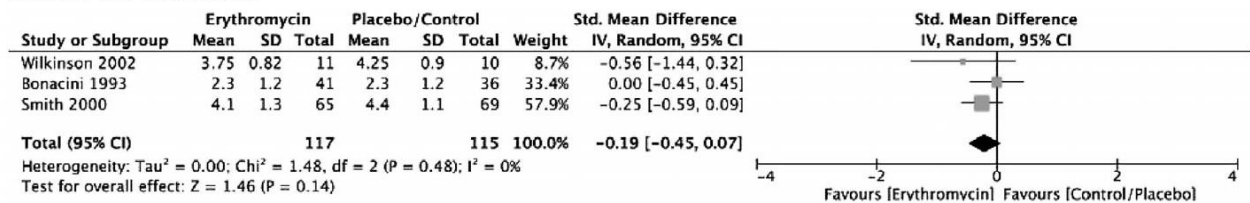


<https://www.endoscopyandsurgery.co.uk/conditions/bowel/small-bowel/>

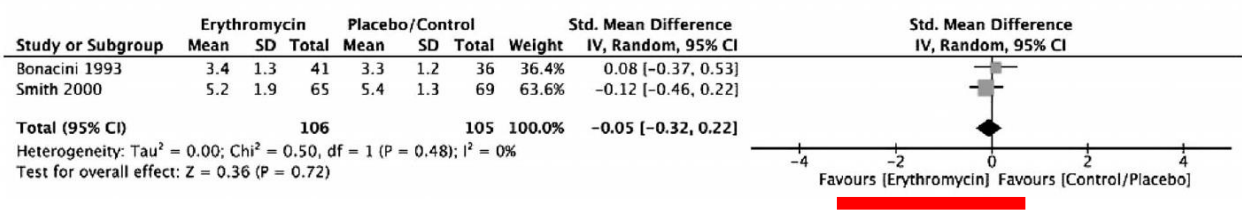
Promotility agents for the treatment of ileus in adult surgical patients: A practice management guideline from the Eastern Association for the Surgery of Trauma

Nikolay Bugaev, MD, Bishwajit Bhattacharya, MD, William C. Chiu, MD, John J. Como, MD, MPH, Michael W. Cripps, MD, Paula Ferrada, MD, Rondi B. Gelbard, MD, Stephen Gondek, MD, MPH, George Kasotakis, MD, MPH, Dennis Kim, MD, Caleb Mentzer, DO, Bryce R. H. Robinson, MD, Edgardo S. Salcedo, MD, and D. Dante Yeh, MD, Miami, Florida

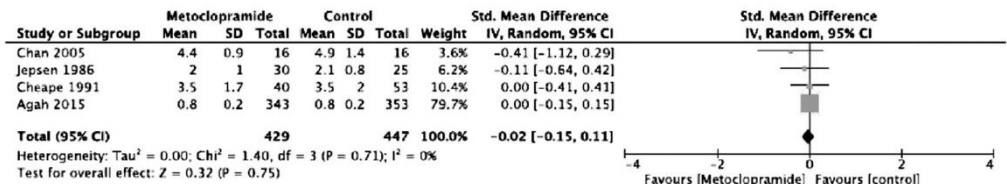
A Time to the first flatus



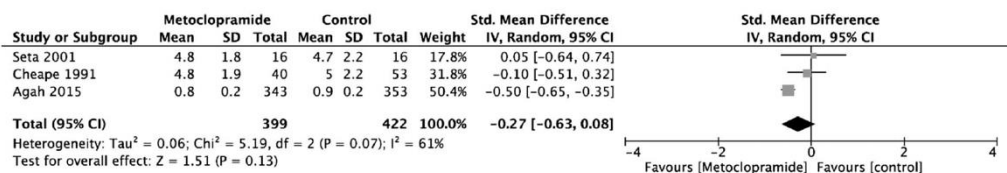
B Time to the first bowel movement



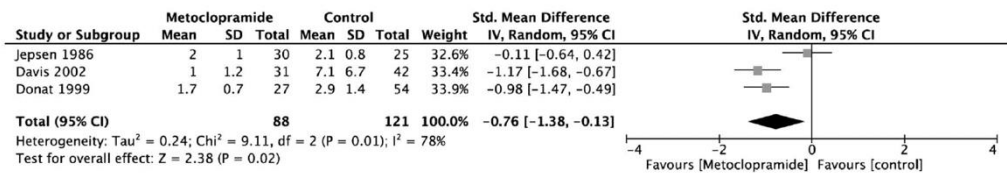
A Time to the first flatus



B Time to the first bowel movement



C Duration of nasogastric tube



D Rate of nasogastric tube reinsertion

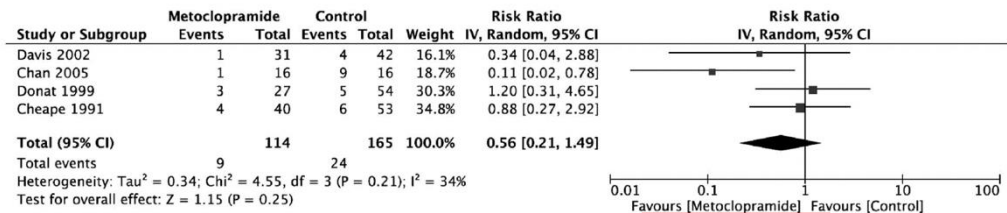


Figure 2. Metoclopramide. A, Time to the first flatus. B, Time to the first bowel movement. C, Duration of NGT. D, Rate of NGT reinsertion. E, Attainment of goal enteral feeding. F, Hospital LOS.

The effect of gum chewing on postoperative ileus in open colorectal surgery patients: A review

Roberta Sammut¹ , Josef Trapani² , Justin Deguara³ and Valeria Ravasi⁴

Journal of Perioperative Practice
0(0) 1–8
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Chiclete x Íleo PO

Sammut et al.

7

Table 3. Risk of bias × other bias – limitations in measuring outcomes (eg: patient-reported outcomes and imprecision of outcome measurement were considered).

	Random sequence generation	Alloc. concealment	Blinding participants and personnel	Blinding outcome assessment	Incomplete outcome data	Selective reporting	Other bias
Mahmoud and Mohammad (2018)	High risk	High risk	High risk	High risk	Low risk	Low risk	High risk
Yang et al (2018)	Low risk	Low risk	High risk	High risk	Unclear risk	Low risk	High risk
Kalyanwat et al (2018)	High risk	Unclear risk	High risk	High risk	Unclear risk	Low risk	High risk

Positivos

Negativo

Featured Article

Coffee administration to promote return of bowel function after small bowel resection: A randomized, controlled trial[☆]

Conor Holland^{a,*}, Lynn Shaffer^b, Elliot Dobkin^{a,1}, Jamie Hall^{a,2}

^a Graduate Medical Education, Mount Carmel Grove City, 5300 North Meadows Drive, Grove City, OH, 43123, USA

^b Mount Carmel Research Institute, 5300 North Meadows Drive, Grove City, OH, 43123, USA

Table 3

Outcomes by Study Group: Coffee vs. Water.

Outcomes	Coffee n (%) or Mean (SD)	Water n (%) or Mean (SD)	p-value
End of Small Bowel Resection to NG tube removal (days)*	3.3 (1.2) Median = 3.4	4.6 (2.3) Median = 4.0	0.0021
End of Small Bowel Resection to first flatus (days)*	3.0 (1.4) Median = 3.1	3.7 (1.7) Median = 3.6	0.0379
End of Small Bowel Resection to first BM (days)*	4.2 (1.5) Median = 4.0	4.8 (1.9) Median = 4.6	0.0788

Menor tempo para Flatus

Menor tempo para Sacar SNG

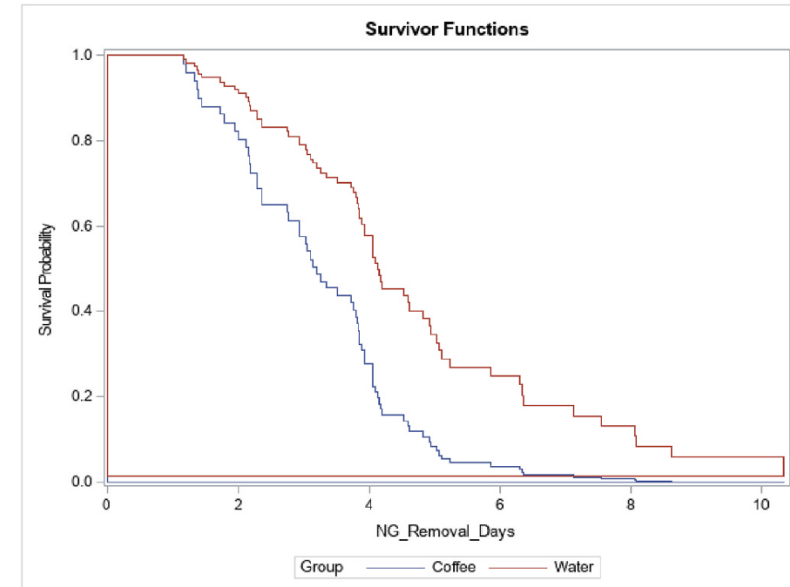
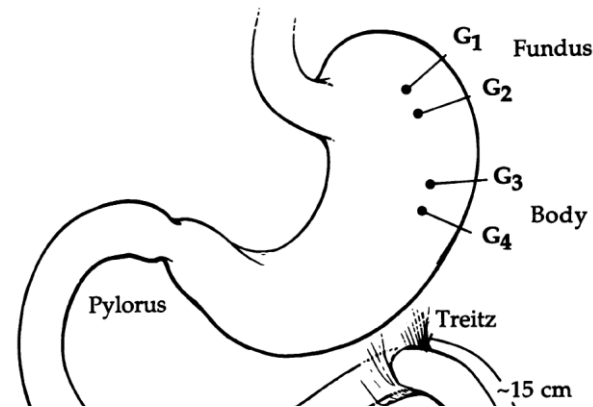


Fig. 1. Survival Curves for Time to NG Tube Removal: Coffee vs. Water Group (p = 0.0021 by log rank test).

Café x Íleo
Pós Enterectomia

The Effect of Ambulation on Recovery from Postoperative Ileus

Ann. Surg. • December 1990



postoperative days 3 or 4. The data suggest that ambulation as a means to help resolve postoperative ileus and its accompanying cramps and bloating may be more perceived than real.

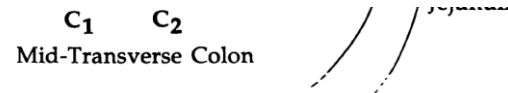


FIG. 1. Placement of seromuscular bipolar myoelectric recording wires in patients without a Roux limb.

Ambulation appears to have no overall effect on promoting an early recovery of normal gastrointestinal myoelectric activity. These results are contrary to our hypoth-



Safety and Efficacy of Intermittent Bolus and Continuous Infusion Neostigmine for Acute Colonic Pseudo-Obstruction

Lucas W. Smedley, PharmD ^{1,2,3}, Dana B. Foster, PharmD, BCCCP^{1,2,3}, Colleen A. Barthol, PharmD, BCPS^{1,2,3}, Reed Hall, PharmD, BCPS^{1,2,3}, and G. Christina Gutierrez, PharmD, BCPS^{1,2,3}

Neostigmine Bolus x BIC

N 75

Retrospective

Table 3. Primary and Secondary Outcomes.

	Bolus	Infusion	P Value
Initial clinical response, n (%)	23 (62.2)	31 (81.6)	.06
Reduction in bowel diameter, n (%)	15 (40.5)	28 (73.7)	.0037
Time to BM in hours, median (IQR)	1.4 (0.975-11.1)	3.5 (2-11.43)	.048
Second neostigmine course after treatment, n (%)	12 (32.4)	10 (26.3)	.56
Colonic decompression after treatment, n (%)	25 (67.6)	15 (39.5)	.015
Surgical intervention after treatment, n (%)	0 (0)	2 (5.3)	.49
ICU length of stay in days, median (IQR)	14.3 (6.95-18.75)	14.5 (8.7-17.45)	.79
Hospital length of stay in days, median (IQR)	25 (11-39.75)	18 (15-25.5)	.46
In-hospital mortality, n (%)	4 (10.8)	2 (5.3)	.43
Bradycardia, n (%)	5 (13.5)	15 (39.5)	.011
Required atropine, n (%)	4 (10.8)	2 (5.26)	.43

Abbreviations: BM, bowel movement; IQR, interquartile range.

Importance of intraoperative oliguria during major abdominal surgery: findings of the Restrictive Versus Liberal Fluid Therapy in Major Abdominal Surgery trial

Paul S. Myles^{1,2,*}, David R. McIlroy^{1,2}, Rinaldo Bellomo^{3,4} and Sophie Wallace^{1,2}

¹Department of Anaesthesia and Perioperative Medicine, Alfred Hospital, Melbourne, Australia, ²Department of Anaesthesia and Perioperative Medicine, Monash University, Melbourne, Australia, ³Department of Intensive Care, Austin Hospital, Heidelberg, Australia and ⁴Department of Surgery, University of Melbourne, Heidelberg, Australia

Oligúria X REMIT

Oígúria no IOP >> 75% não resulta em IRA

Factor	AKI	RR (95% CI)	P-value	Treatment group† interaction
All patients (n=2354)				
Any oliguria				
Yes (n=864)	220 (26)	1.29 (1.14–1.45)	<0.001	<0.001
No (n=1490)	274 (18)	Reference		

the positive predictive value of oliguria was 25.5%



ORIGINAL ARTICLE

Acute kidney injury after liver surgery: does postoperative urine output correlate with postoperative serum creatinine?

Gaëtan-Romain Joliat, Ismail Labgaa, Nicolas Demartines & Nermin Halkic

Department of Visceral Surgery, Lausanne University Hospital CHUV, Lausanne, Switzerland

**Correlação fraca entre
Diurese 1 PO e Creatinina 2 PO**

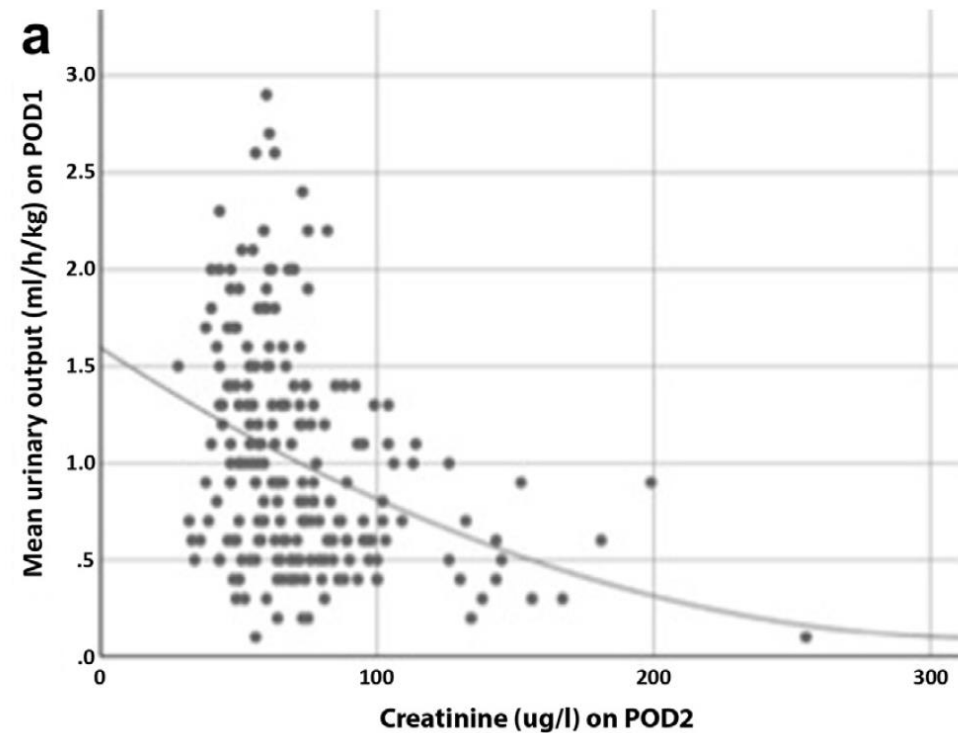


Figure 1 Scatter plots. **a.** Creatinine levels on POD 2 and mean urine output on POD 1 for the entire cohort ($\rho = -0.34$, $p < 0.001$).





Contents lists available at [ScienceDirect](#)

Journal of Critical Care

journal homepage: www.journals.elsevier.com/journal-of-critical-care



Postoperative oliguria after intermediate and high-risk surgeries in critical care, A cohort analysis[☆]

Table 3

Outcomes according to the presence of postoperative oliguria.

Rod
Brur
Ferr



Contents lists available at [ScienceDirect](#)

Journal of Critical Care

journal homepage: www.journals.elsevier.com/journal-of-critical-care



Res

147
the
first

Should diuresis remain a diagnostic criterion for surgery associated acute kidney injury? Revisiting the role of perioperative oliguria



ICU Mortality - n (%)	21 (2,1)	14 (2,7)	0.48	1.27 (0.65–2.47)
Hospital Mortality - n (%)	22 (2,2)	15 (2,9)	0.42	1.29 (0.67–2.47)

Categorical variables are presented as frequency (N) and %. Continuous parametric variables are presented as mean and standard deviation and non-parametric variables are described as median and [IQR 25–75 %]. AKI: Acute Kidney Injury; RRT: Renal Replacement Therapy; ICU: Intensive Care Unit;

DRENOS

- Volume
- Aspecto
- Lab (Ht ?)
- Prós x Contras



Table 3 Main characteristics of published studies on the value of prophylactic drainage for liver resection.

Authors and references	Year	Type of study	Number of patients	Main results
Belghiti et al. [27]	1993	Prospective randomized	42 Drainage group 39 No drainage group	No significant difference in mortality or complication rate. More (and more often infected) collections in drainage group
Fong et al. [28]	1996	Prospective randomized	60 Drainage group 60 No drainage group	No significant difference in postoperative course or in duration of hospital stay
Liu et al. [1]	2004	Prospective randomized in patients with chronic hepatic disease	52 Drainage group 52 No drainage group	No significant difference in mortality. More morbidity in drainage group (more SSI). Longer duration of hospital stay in drainage group
Fuster et al. [29]	2004	Prospective randomized in cirrhotic patients	20 Drainage group 20 No drainage group	Fewer ascites leaks, local complications and shorter duration of hospital stay in drainage group
Sun et al. [30]	2006	Prospective randomized	60 Drainage group 60 No drainage group	More SS complications in drainage group. No significant difference in duration of hospital stay
Petrowsky et al. [9]	2004	Meta-analysis of 3 randomized trials	154 Drainage group 150 No drainage group	Fewer deep OSI in no drainage group (NS). No significant difference in rate of biliary collections or pulmonary complications
Gurusamy et al. [5]	2007	Meta-analysis of 5 randomized trials	234 Drainage group 231 No drainage group	No significant difference in mortality, or morbidity (re-intervention, collections, wound disruption, abscess biliary leak, SSI, pulmonary infection). No significant difference in need for further drainage. More ascites leak in drainage group

Is there still a need for prophylactic intra-abdominal drainage in elective major gastro-intestinal surgery?

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SS: surgical site; SSI: surgical site infection; OSI: organ site infection; NS: non-significant.

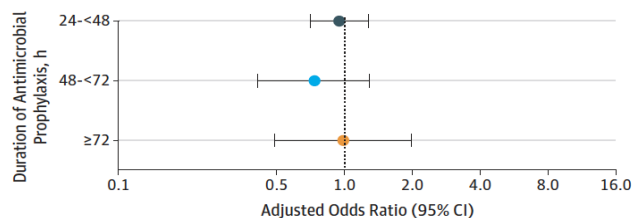
FERIDA OPERATÓRIA

- Inspeção diária
- Abaulamentos
- Secreção
- Deiscência

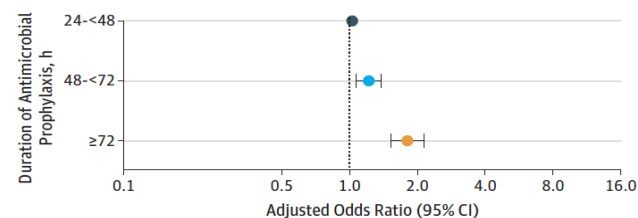


ATB PROFILAXIA

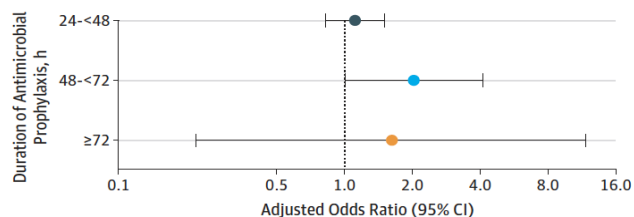
A SSI after cardiac surgery



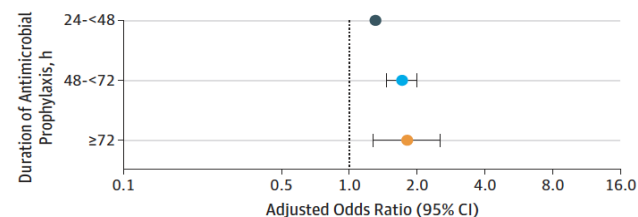
E Acute kidney injury after cardiac surgery



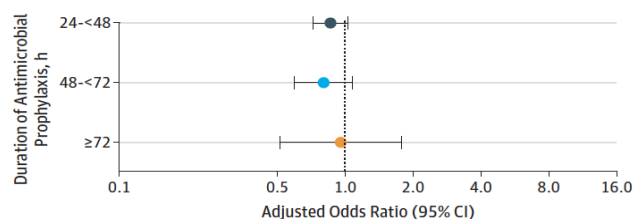
B SSI after orthopedic surgery



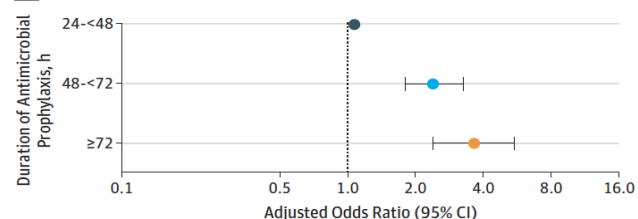
F Acute kidney injury after noncardiac surgery



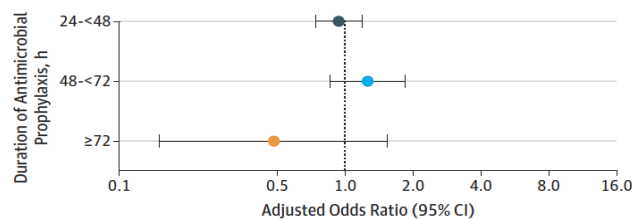
C SSI after colorectal surgery



G *Clostridium difficile* infection after any surgery



D SSI after vascular surgery



JAMA Surgery | Original Investigation

Association of Duration and Type of Surgical Prophylaxis With Antimicrobial-Associated Adverse Events

Westyn Branch-Elliman, MD, MMSc; William O'Brien, MS; Judith Strymish, MD; Kamal Itani, MD; Christina Wyatt, MD; Kalpana Gupta, MD, MPH



Trial of Short-Course Antimicrobial Therapy for Intraabdominal Infection

R.G. Sawyer, J.A. Claridge, A.B. Nathens, O.D. Rotstein, T.M. Duane, H.L. Evans, C.H. Cook, P.J. O'Neill, J.E. Mazuski, R. Askari, M.A. Wilson, L.M. Napolitano, N. Namias, P.R. Miller, E.P. Dellinger, C.M. Watson, R. Coimbra, D.L. Dent, S.F. Lowry,* C.S. Cocanour, M.A. West, K.L. Banton, W.G. Cheadle, P.A. Lipsett, C.A. Guidry, and K. Popovsky, for the STOP-IT Trial Investigators†

ANTIBIÓTICOS

Após intervenção pra resolução de foco

2 dias após resolução clínica (Max 10)

X

4 dias

Infecção ou Morte

Table 2. Primary and Major Secondary Outcomes.*

Variable	Control Group (N = 260)	Experimental Group (N = 257)	P Value
Primary outcome: surgical-site infection, recurrent intraabdominal infection, or death — no. (%)	58 (22.3)	56 (21.8)	0.92

DÚVIDAS ?

